Congestive Heart Failure – Section Summary

ADHF • Cardiogenic shock & MCS • Valves • Constriction • LVAD

Critical Care Board Review • Section 8 • MCQ-Oriented

1. Acute Decompensation

- Low-dose dopamine does **not** help renal function
- In-hospital mortality predictors: low SBP, **high creatinine/BUN**
- Diuresis: **IV furosemide** ≥ home dose (80–100 mg bid)
- HFpEF: **no therapy proven to reduce mortality**
- Wide QRS → **CRT** target

2. Shock & Mechanical Support

- LV failure, preserved RV → **Impella**; biventricular/hypoxia → **VA-ECMO**
- Advanced HF non-transplant → **LVAD destination therapy**
- IABP contraindicated: **severe AI** → remove
- Peripartum cardiomyopathy → ACEi (after delivery) + β-blocker
- RHC to define haemodynamics

3. Valves, Pericardium, AF

- AF > 48 h → **TEE-guided cardioversion**
- M/W-shaped RA waveform → **constriction** (TB → CT for calcification)

4. LVAD

- MAP target **70–80 mmHg** (Doppler)
- Haemolysis, ↑ LDH, power spikes → **pump thrombosis**

5. High-Yield MCQ Pearls

- ☑ Dopamine doesn't protect kidneys
- ☑ IABP contraindicated in AI

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